

Health and human rights

Migration, human rights, and health

On Dec 17, 2003, on the eve of International Migrants Day, WHO¹ are publishing a report on the health of migrants and human rights that emphasises the need for public health interventions to be done within a human rights framework. On Oct 17, 2003, after a trial lasting 7·5 years, Irene Fernandez, a prominent activist for the rights of migrant workers and people infected with HIV/AIDS, was sentenced to 12 months in prison for reporting on harsh conditions and abuse in migrant detention centres in Malaysia.² These two events highlight the gap between, on the one hand, international public-health policies and programmes and on the other, migrants and the people who work with them. In this article, we review the WHO report from the perspective of Asian non-governmental organisations (NGOs). We highlight the need to address health and migration from a human-rights based public-health perspective, and discuss the challenges in integrating migrant voices with the development of national and regional health agendas.

About 175 million people (2·9% of the world's population) live outside their countries of origin (table).³ This number includes only people with documents; the total number of migrants is probably much higher.

Migrants have many names. Voluntary migration is often compared with so-called forced migration, a term which includes refugees (27·4 million), internally displaced people (30 million), and people displaced by disasters, including famine and development projects; for instance, 10 million people are displaced by dam projects every year.⁴ A further distinction is made between regular (or documented) and irregular (or undocumented) migrants. In some countries the term "illegal" is inappropriately used for an undocumented migrant. Although the difference between regular and irregular migrants may be clear for legislators, migrants themselves—being dependent on intermediaries in the migration process—are often unaware of their status.

Permanent migrants also differ from temporary migrant workers. Rich countries are mostly interested in

migrants as a labour source, and allow them short-term contracts for the so-called 3D jobs (dirty, dangerous, and degrading). These are roles that are unpopular in the host country: cleaning jobs, domestic work, construction work, farm work, and jobs in the entertainment sector (often a euphemism for sex work) (figure).

Migrants are willing to accept jobs below their level of training. In the Philippines, doctors retrain as nurses because they can receive better pay overseas as a nurse than as a doctor in their home country. For example, in 2003, 2000 doctors—2·9% of all doctors practising in the Philippines—enrolled in nursing schools and courses for this reason. Despite increasing rates of skilled migration, migrant workers are most often recruited from the most vulnerable populations in poor countries. Sometimes it is difficult to decide where recruiting stops and trafficking starts.

The number of trafficked people is estimated to be between 700 000 to 4 million every year.⁵ Trafficking agents are often also involved in recruiting networks. Migrants (and their families) pay recruiters large amounts of money to find jobs and organise temporary migration, often leading to dependency and heavy debt. Once they arrive in their new country, migrants' wages are often lower than they were expecting, making it impossible to repay their intermediary. Returning home

becomes practically impossible. Thus, there are large migrant populations who are frustrated because of loneliness, adaptation problems, discrimination, and stigmatisation. Furthermore, destination countries often provide a single-entry visa for migrants who must then emigrate without families or partners, thus taking away their usual support mechanisms.

Migrants, especially those without documents, generally have limited access to health facilities and resources in their new countries. They live in transnational communities with parallel identities and run high health risks.⁶ Most states view migrants as a health threat. Yet, it is migration itself that can cause health threats to migrants, which in turn causes risks for communities in receiving countries. Public-health programmes usually mirror official government policies, and public-health processes can be used to control migration. In 1998, the United Arab Emirates screened their entire population and repatriated workers who tested positive for HIV/AIDS. In 60 countries, HIV testing is mandatory before migrants are allowed to enter the country. The Malaysian Immigration Act decrees that migrant workers submit to pre-employment medical examinations. Health-care workers report a wide range of diseases under the Prevention and Control of Infectious Diseases Act, which can lead to migrants with

	Immigrants in the country (×1000)	Proportion of total population (%)
More-developed regions	10 4119	8·7
Australia/New Zealand	5555	24·2
North America	40 844	13·0
Western Europe	18 836	10·3
Northern Europe	7453	7·8
Eastern Europe	25 812	8·2
Southern Europe	4999	3·5
Japan	1620	1·3
Less-developed regions	70 662	1·5
Africa	16 277	2·1
Asia	49 781	1·4
Western Asia	19 480	10·3
South-central Asia	20 407	1·4
East Asia	5769	0·4
Southeast Asia	4126	0·8
Latin America and the Caribbean	5944	1·1
Least-developed regions*	10 458	1·6
World	174 781	2·9

*Includes 49 countries. Data from UN Population Division.³

Migrants around the world

Rights were not granted to include this image in electronic media. Please refer to the printed journal.

Migrant workers

Clockwise from top left: a migrant worker in a northwest Indian salt pan; Myanmar workers in Bangkok, Thailand; Jamaican farmworkers in Vermont, USA; and foreign domestic workers in Hong Kong protest against decreased wages.

HIV infection and other notifiable diseases being deported. Increasingly, European countries and the USA use a migrant's undocumented status as a way to control migration by denying them access to health care and other facilities.

Migrant communities are mobile. Construction workers follow a highway or pipeline as it is constructed, sex workers are often moved to brothels around the country, and farmworkers move with the seasons. Conventional public-health programmes are often directed at sedentary communities and, thus, are not very successful for migrants.

All these reasons explain why most innovative work in migrant health is done by community based organisations (CBOs) and national NGOs that work against authorities to develop alternative health programmes for migrants. **Instead of controlling migration, these organisations try to improve migrants' health conditions, and create an enabling environment to empower migrants.** To deliver these programmes in a human rights framework needs an **empowering and holistic approach** that crosses borders. Regional programmes are needed that target people before they migrate, those who have migrated, and also returnees. Official transnational public-health programmes are rare, and NGOs have taken the lead in this area to experiment with innovative approaches. For instance, Asian NGOs have joined in a network (CARAM or Coordination of Action Research on AIDS and Migration)

that meets regularly to develop interventions, and does advocacy work to have input to policies at a regional level.⁷

The WHO report¹ acknowledges the magnitude of the problems faced by migrants and quite rightly underlines the fact that the **combination of poor attitudes towards migrants in most countries, discrimination, stigmatisation, and limited access to facilities can have a serious impact on migrants' health.** Therefore, the human rights situation of migrants has to be addressed if their health status is to improve. The WHO report will be useful for workers in public health and contains a good overview of the current situation and existing human rights laws and conventions.

However, we would encourage WHO and other UN agencies to engage more directly with migrants themselves. And, we would caution against placing too much confidence in the available human rights tools and UN declarations. For example, it took 13 years to get the needed signatures of states to ratify the most important of these, the International Convention on the Protection of the Rights of All Migrant Workers and Members of their Families of 1990. Receiving countries were reluctant to sign, and sending countries tried to protect their labour bargaining positions.⁸ The Convention contains many ambiguous formulations such as that in Article 8, which says that migrants and their families have the right to leave any state and "shall not be subject to any restrictions except

those that are provided by law to protect . . . public health". Thus the possibility remains that purported public-health reasons can be used to restrict people's movement and turn health care into an instrument that can be used against migrants.

UN organisations are not an independent global government; they are financially dependent on member countries. One cannot discuss the health and human rights of migrants without acknowledging the political realities of migration. Until UN organisations have the moral authority to deal with this reality, their advice will be targeted at a political vacuum, rather than at real people in the real world. Moreover, **for public-health initiatives for migrants to be beneficial, they need to take into account the reality of inequalities between countries and people,** where policymakers have an interest in transfer of cheap labour without responsibilities, and where unscrupulous intermediaries abuse the lack of legislation to protect migrants. There is much left to do by people, such as Irene Fernandez, who are brave enough to stand up for the rights of migrants.

*Ivan Wolffers, Sharuna Verghis, Malu Marin

*Health Care And Culture, Vrije Universiteit Medical Centre, Amsterdam, Netherlands (IW); CARAM Secretariat, Kuala Lumpur, Malaysia (SV); Action for Health Initiatives (ACHIEVE), CARAM-Philippines, Manila, Philippines (MM)
(e-mail: i.wolfers@vumc.nl)

- 1 WHO. International migration, health, and human rights. Geneva: WHO (in press).
- 2 Tenaganita. Abuse, torture, and dehumanized treatment of migrant workers at detention camps. Kuala Lumpur: Tenaganita, 1995.
- 3 Department of Economic and Social Affairs. International Migration Report 2002. http://www.un.org/esa/population/publications/itmmig2002/WEB_migration_wallchart.xls (accessed Dec 10, 2003).
- 4 Hunt P. Report of the UN Special Rapporteur on the right of everyone to the enjoyment of the highest attainable standard of physical and mental health. Geneva: 2003.
- 5 USAID Office of Women in Development. Trafficking in persons: USAID's response. Washington: USAID, 2001.
- 6 Wolffers I, Fernandez I, Verghis S, Vink M. Sexual behaviour and vulnerability of migrant workers for HIV infection. *Cult Health Sex* 2002; 4: 459-73.
- 7 Coordination of Action Research on AIDS and Migration (CARAM). Migrants, health, and the abuse of human rights. Kuala Lumpur: CARAM (in press).
- 8 Piper N, Iredale R. Identification of the obstacles to the signing and ratification of the UN convention on the protection of the rights of all migrants 1990—the Asia Pacific perspective. UNESCO, 2003.